

UnitedHealthcare

CLINICAL COVERAGE DETERMINATION GUIDELINE

Advanced Imaging: Lumbar Spine MRI

Policy: POL-RAD-501 | Effective: 2026-01-01 | Radiology

COVERED PROCEDURES

- 72148 - MRI lumbar spine without contrast
- 72149 - MRI lumbar spine with contrast
- 72158 - MRI lumbar spine without contrast followed by with contrast

DIAGNOSIS CODES

- M54.5 - Low back pain
- M54.16 - Radiculopathy, lumbar region
- M54.17 - Radiculopathy, lumbosacral region
- M51.16 - Intervertebral disc disorder with radiculopathy, lumbar
- M51.17 - Intervertebral disc disorder with radiculopathy, lumbosacral
- G55 - Nerve root and plexus compressions
- M47.816 - Spondylosis with myelopathy, lumbar
- M47.817 - Spondylosis with myelopathy, lumbosacral

MEDICAL NECESSITY CRITERIA

All of the following must be documented for approval:

CRIT-1

Conservative therapy (physical therapy, NSAIDs, activity modification) for minimum 6 weeks with documented failure
Threshold: 6 weeks

CRIT-2

Neurological symptoms present: radiculopathy, numbness/tingling, weakness, or bowel/bladder dysfunction
Threshold: true

CRIT-3

Red flag symptoms absent OR if present, urgent/emergent pathway applies (cauda equina, progressive neuro deficit, suspected malignancy)
Threshold: screening

CRIT-4

No MRI of the same region within the past 12 months unless new symptoms documented
Threshold: 12 months

URGENT BYPASS

If cauda equina syndrome suspected (bilateral leg weakness, saddle anesthesia, bowel/bladder dysfunction), immediate MRI is authorized without conservative therapy requirement.

DENIAL REASONS

- Insufficient conservative therapy duration
- No documented neurological findings on exam
- Recent MRI within 12 months without new symptoms
- Non-specific low back pain without radicular features